Family Violence / Abuse

UP-21 | Universal Patient Care

Quick flow

01 Scene Safe? [EMT / AEMT / Paramedic]

Immediate threat • weapons • escalating behavior • request law enforcement or additional resources

02 Treat Immediate Threats [EMT / AEMT / Paramedic]

XABCDE • hemorrhage • airway • shock • pain • strangulation • age-appropriate complaint protocol

03 Create Privacy When Safe [EMT / AEMT / Paramedic]

Separate patient from others • qualified interpreter • never use suspected aggressor or child as interpreter

04 Patient Younger Than 18? [EMT / AEMT / Paramedic]

Reasonable suspicion: immediately report to DCS • hospital or law-enforcement notice is not a substitute

05 Elderly / Vulnerable Adult? [EMT / AEMT / Paramedic]

Immediately report abuse, neglect, or exploitation to APS • suspected sexual offense: APS + local law enforcement

06 Competent Adult Preference [EMT / AEMT / Paramedic]

No automatic police report over objection unless statutory exception applies

07 Preserve + Document [EMT / AEMT / Paramedic]

Exact words • objective findings • who was present • scene conditions • evidence handled • report details

08 Safe Disposition [EMT / AEMT / Paramedic]

Private handoff • required reports • do not leave child or vulnerable adult in an unsafe plan

Overview

- Use this protocol for suspected intimate-partner or family violence, child abuse or neglect, elderly or vulnerable-adult abuse or exploitation, caregiver abuse, or human trafficking.
- Immediate patient care and scene safety come first. Provide private, trauma-informed, nonjudgmental care and use the appropriate trauma, medical, pediatric, behavioral, obstetric, or sexual-assault protocol concurrently.
- EMS recognizes, treats, protects, documents, and reports. EMS does not investigate, confront a suspected offender, or attempt to prove that abuse occurred.

Provider-Level Actions

All providers share responsibility for recognition, immediate safety, compassionate care, objective documentation, and required reporting.

EMT

- Treat immediate life threats and request law enforcement or additional resources when danger is present.
- When safe, speak with the patient privately. Use calm, simple, nonjudgmental, non-leading questions and avoid repeated questioning.
- Do not use the suspected aggressor or a child as interpreter. Request a qualified interpreter when needed.
- Preserve potential evidence, document objective findings and the patient's exact statements, and ensure the required reporting pathway is completed.

AEMT

- Perform all EMT actions. Provide authorized vascular access, fluid, medication, and monitoring care for associated illness or injury.

- Reassess for occult injury, shock, overdose, strangulation, pregnancy-related concerns, and medical causes of altered behavior.
- Do not delay transport or a required report to obtain a detailed history or complete documentation on scene.

Paramedic

- Perform all prior care. Lead assessment for serious occult injury, strangulation, head injury, cardiac or neurologic complications, pregnancy emergency, overdose, and need for specialty destination.
- Contact Medical Control for questionable capacity, high-risk refusal, conflict between patient safety and caregiver or custodial demands, or an unsafe proposed disposition.
- Provide a private receiving-facility handoff and document the agency contacted, recipient, time, report or reference number, and any safety plan.

Tennessee Reporting Pathways

Reporting duties depend on patient age, vulnerability, suspected offense, injury, capacity, and transport disposition.

EMT

- Child younger than 18: when reasonable suspicion exists, immediately report suspected abuse, neglect, exploitation, or child sexual abuse to Tennessee DCS at 1-877-237-0004. Ensure the report is made; hospital or law-enforcement notification alone does not replace the duty.
- Elderly or vulnerable adult: immediately report suspected abuse, neglect, or exploitation to Tennessee APS at 1-888-277-8366.
- Suspected rape, aggravated rape, sexual battery, or aggravated sexual battery of an elderly or vulnerable adult: report to both APS and local law enforcement.
- Competent adult intimate-partner violence: do not automatically release identifying information to law enforcement over the patient's objection unless a statutory exception applies.
- Life-threatening injury, strangulation, or injury involving a knife, firearm, or other deadly weapon follows the mandatory injury-reporting pathway. For an injury covered by Tennessee Code §38-1-101, complete and document the EMS notification step: when transporting, privately tell the receiving physician or emergency-department staff the suspected cause of injury; when not transporting, report the result of the call to the 911 center through the approved agency channel. This does not replace separate required child-protection, adult-protection, or other legally required reports. Follow the approved safeguarding policy when reporting duties overlap.

AEMT

- Support immediate reporting and continue patient care. Do not assume another responder or the receiving hospital has completed the clinician's duty unless confirmation is obtained and documented.

Paramedic

- Resolve uncertainty through the agency safeguarding chain, Medical Control, and law-enforcement or protective-service consultation without delaying an immediately required report.

Indications

- Patient disclosure of violence, abuse, neglect, exploitation, coercive control, trafficking, or fear of a household member, caregiver, partner, or other person.
- Injury pattern, history, behavior, living condition, caregiver interaction, or scene finding that creates reasonable suspicion for abuse, neglect, exploitation, or trafficking.
- Any child, elderly adult, or vulnerable adult for whom a mandatory safeguarding report may be required.

Contraindications

- Do not confront or accuse the suspected offender, conduct an independent investigation, or delay emergency treatment to collect evidence.

- Do not promise secrecy when a mandatory report may be required.

Assessment | Safety and immediate medical assessment

- Identify immediate threats, weapons, escalating behavior, unsafe persons, children or dependents at risk, and the need for law enforcement or additional resources.
- Perform XABCDE and assess for hemorrhage, head injury, strangulation, chest or abdominal injury, fracture, pregnancy complications, overdose, intoxication, neglect, dehydration, hypothermia, and psychiatric emergency.
- When safe, separate the patient from household members, caregivers, or a suspected aggressor and ask whether the patient feels safe returning to the current environment.

Assessment | Trauma-informed inquiry

- Explain that questions are asked because violence and abuse affect health and safety.
- Use broad, non-leading prompts such as: 'Tell me what happened,' 'Has anyone hurt or threatened you?' and 'Do you feel safe where you live?'
- Do not pressure a patient to disclose, repeat a detailed account, identify an offender, or make an immediate law-enforcement decision unless a mandatory report applies.
- Use the patient's own words in quotation marks when possible and distinguish statements from EMS observations.

Assessment | Possible strangulation

- Assess voice change, dysphagia, dyspnea, neck pain or tenderness, petechiae, loss of consciousness, incontinence, seizure, confusion, focal neurologic symptoms, and memory gaps.
- Visible neck injury is not required for serious airway or vascular injury. Strongly recommend emergency-department evaluation even when initial findings are minimal.
- Treat strangulation as a high-risk mechanism and follow the mandatory injury-reporting pathway.

Treatment steps

- For an injury covered by Tennessee Code §38-1-101, complete and document the EMS notification step: when transporting, privately tell the receiving physician or emergency-department staff the suspected cause of injury; when not transporting, report the result of the call to the 911 center through the approved agency channel. This does not replace separate required child-protection, adult-protection, or other legally required reports. Follow the approved safeguarding policy when reporting duties overlap.
- Ensure scene safety, request law enforcement or additional resources when danger is present, and treat immediate life threats without delay.
- Create privacy when safe, use a qualified interpreter, and use trauma-informed, non-leading questions only to the extent needed for medical care, safety, and reporting.
- Treat associated illness or injury under the appropriate Claiborne protocol and preserve evidence when doing so does not compromise care.
- For reasonable suspicion involving a patient younger than 18, immediately report to DCS at 1-877-237-0004 and document the report.
- For suspected abuse, neglect, or exploitation of an elderly or vulnerable adult, immediately report to APS at 1-888-277-8366. For a suspected sexual offense against an elderly or vulnerable adult, also notify local law enforcement.
- For a competent adult experiencing intimate-partner violence, respect the patient's decision regarding police involvement unless a statutory injury or other mandatory-reporting exception applies.
- Strongly recommend emergency-department transport after strangulation, significant injury, altered mental status, pregnancy concern, overdose, suicidal risk, or an unsafe disposition.

- A competent adult may refuse transport when capacity is intact, but refusal does not cancel a mandatory report. Do not leave a child or vulnerable adult solely under an unsafe caregiver refusal; request law enforcement, DCS or APS, Medical Control, and the agency safeguarding chain.
- Provide a private handoff and document objective findings, exact statements, consent, capacity, evidence handling, safety concerns, reporting decisions, notifications, recipient, time, and reference number.

Warnings / precautions

- Patient safety and emergency medical care take priority over evidence preservation.
- A patient's refusal to disclose details or involve law enforcement does not imply that abuse did not occur.
- Do not use a suspected offender, coercive caregiver, or minor child as interpreter.
- Age alone does not remove an adult patient's autonomy. Apply the statutory elderly or vulnerable-adult criteria and assess capacity individually.
- Do not leave a child or vulnerable adult in an unsafe environment solely because a caregiver refuses transport or minimizes the concern.

Clinical pearls

- EMS needs reasonable suspicion - not proof - to initiate a mandatory child or vulnerable-adult report.
- Abuse may coexist with medical illness, disability, cognitive impairment, substance use, pregnancy, or financial dependence.
- Objective documentation and the patient's exact words are more useful than conclusory labels.
- Strangulation can produce delayed airway, vascular, and neurologic complications despite few or no visible findings.

Special populations | Children and adolescents

- The mandatory child-reporting threshold is younger than 18 years, even though Claiborne pediatric medication dosing generally applies to patients younger than 16 years.
- Use developmentally appropriate, non-leading questions. Do not repeatedly interview the child or ask the child to confront the suspected offender.

Special populations | Elderly and vulnerable adults

- Assess decision-making capacity individually. Age, disability, or dependence does not automatically remove the right to participate in care decisions.
- Report suspected abuse, neglect, or exploitation to APS and use both APS and local law enforcement for suspected qualifying sexual offenses.

Related pathways / contacts

Call Tennessee DCS - Child Abuse Hotline: 1-877-237-0004

Call Tennessee APS - Adult Protective Services: 1-888-277-8366

Source and references

Tennessee Code Annotated §§ 37-1-403 and 37-1-605 - child abuse and child sexual abuse reporting. | Tennessee Department of Children's Services - Child Abuse Hotline and mandated-reporter guidance. | Tennessee Code Annotated § 71-6-103 - Adult Protection Act reporting. | Tennessee Code Annotated § 39-15-509 - reporting abuse or sexual offenses involving elderly or vulnerable adults. | Tennessee Code Annotated § 38-1-101 - reporting of certain injuries and first-responder duties. | Tennessee Department of Human Services - Adult Protective Services. | Claiborne County EMS safeguarding, reporting, evidence-preservation, law-enforcement, destination, and refusal policies.

Revision: October 31, 2026 | Last verified: October 31, 2026